

Standard Health Insurance Claims Policy Manual

1. Covered Procedure Codes

All procedure codes submitted on a CMS-1500 claim form must be valid CPT codes listed in the current AMA CPT codebook. Unlisted or experimental procedure codes require prior authorization. Code 99213 covers office visits for established patients with moderate complexity. Code 99214 covers office visits of moderate-high complexity. Services must be medically necessary and documented in the patient record.

2. Diagnosis Code Requirements

All diagnosis codes must be valid ICD-10-CM codes at the highest level of specificity. Codes must be in effect for the date of service. J06.9 covers acute upper respiratory infection, unspecified. Codes must support medical necessity of services billed. Combination codes should be used when available rather than multiple separate codes.

3. Charge Discrepancies

When the sum of individual service line charges does not equal the total charge in Box 28, the claim will be flagged for manual review. Discrepancies of more than \$0.01 require a corrected claim or written explanation. Arithmetic errors are a common reason for claim delay. The billing provider is responsible for ensuring mathematical accuracy.

4. Signature Requirements

Physician signature must be on file before submission. Box 31 must contain either the physician's handwritten signature or a stamped signature with authorization on file. Electronic signatures are accepted if the provider has a signed authorization agreement. Claims submitted without a valid signature will be rejected and returned.

5. Date of Service Rules

Dates of service must not be in the future. Services rendered more than 12 months prior to submission date may be denied for timely filing. Hospitalization dates in Box 18 must encompass the dates in Box 24. Dates must be formatted as MMDDYYYY with no separators.

6. Appeals Process

Claims denied for any reason may be appealed within 180 days of the denial date. Appeals must include the original claim, the denial notice, and supporting documentation. First-level appeals are reviewed within 30 days. Expedited appeals for urgent care are reviewed within 72 hours. Written notification of appeal decisions is provided.